The Hillingdon Hospitals
NHS Foundation Trust

Escalation Process for COVID-19 Positive Maternity Patients with COVID Related Illnesses (SOP)

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Amendment Record

Version Number	Changes	Author(s)	Date
2.0	Sarah-Jayne Lamb, Consultant Obstetrician	Minor formatting changes, no content changes required. Guideline still required.	2.0

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Aim/Purpose
In general, a pregnant woman who is COVID-19 positive and needs admission for infection-related complications should be admitted on to a medical ward unless she is in labour or needing delivery when she will be transferred to Labour Ward.

A Maternity Patient On A Medical Ward
See Appendix 1

- Should be seen daily by consultant led obstetric team and obstetric input should be available at all times from the on call obstetric team (bleep 5809). A named consultant should be leading the care. The obstetric team is responsible for allocating and informing the named consultant of the patient's admission. The maternity team will also organize fetal monitoring as needed on the medical ward.

- If the medical team considers that delivery will improve the woman's outcome from the cardio-respiratory point of view, then they must contact the on call obstetric consultant so that delivery can be discussed and arranged by the obstetric team, including the neonatal team as required. The decision for a caesarean section in main theatre or Labour Ward (LW) will be agreed at consultant level. The obstetric and midwifery team will follow up the routine postnatal care with regards to surgery and breastfeeding etc. This should be a daily review until discharged (with a minimum of the first three days unless discharged earlier) from an obstetric/midwifery point of view. The patient should also have a postnatal clinic six weeks after discharge with the named consultant obstetrician.

- If the patient is discharged before delivery, the obstetric follow up for these women/birthing people such as growth scans and clinic appointments (with the name consultant) should be arranged by the on call obstetric team. The medical team must therefore inform the on call obstetric registrar on the day of discharge.

Admission to Labour Ward Where There Is No Medical/ICU Bed

In exceptional circumstances only, a maternity patient may need to be admitted to Labour Ward if there is no appropriate medical/ICU bed. This is more commonly as a step down from ICU to the ward (or a patient transfer from a medical ward) (Appendix 2).

This decision must be made at consultant level, with the following communication:

1. The Medical team, Intensive Care Unit (ICU) team and Obstetrician Consultant on call should first do a risk assessment for all the pregnant women/birthing people currently in hospital for COVID-19 related disease so that the least sick pregnant woman should be considered for transfer. All in-patient obstetric patients are on the daily handover list on Labour Ward white board. The Divisional Director (DD) of Unplanned Care and Planned Care should be informed of the decision.
2. A multidisciplinary communication should take place, in regard to the level of medical and nursing care as well as the level of monitoring needed should be discussed.
3. The obstetric consultant will discuss the case with the labour ward (LW) midwife co-ordinator to ensure there is a bed available depending on LW activity and staff skill level.
4. The obstetric consultant will then need to inform the Director of Midwifery (DOM) and Divisional Director of Planned Care with the transfer before accepting the patient.

5. **If accepted**, the obstetric consultant or registrar will inform the on call medical registrar bleep 5807 if the patient is on ICU – the medical registrar may need to liaise with ICU directly for any further information about current medical management.
6. The Labour Ward Coordinator will discuss the care with the ICU nurse/ward nurse in charge.
7. The Labour Ward Coordinator will inform:
    a. the maternity bleep holder on bleep 5763
    b. the clinical site practitioner on mobile 07810528077 or bleep 5838
    c. the critical care outreach team (CCOT) nurse on bleep 5320 (currently daily service 8am-8pm)
    d. the Infection, Prevention and Control Team on telephone 3767/3648
8. The medical registrar will inform the on call medical consultant that the patient is being moved to LW if this is out of hours. During the day this will be done by the medical parent team. The medical team (or the DD of Unplanned Care) will inform the obstetric team of the Named Medical Consultant leading in the patient's care.

Transfer is done with full acknowledgement that a minimum of a **daily review by both the medical team and the CCOT** together with the obstetric team (both obstetricians and anaesthetists) will be carried out.

A multidisciplinary handover between the ICU, medical and Labour Ward teams should be carried out including the observations expected, the treatment and tests needed. This is likely to include as a minimum:

- hourly observations including O2 saturations, target saturations must be specified
- hourly fluid balance
- 4-6 hourly paracetamol
- VTE prophylaxis
- daily blood tests
- daily physiotherapy bleep 5664

Maternity patients will always be for full escalation of care as documented on a Treatment Escalation Plan (TEP). Specific instructions must be documented by the transfer team regarding any specific medication or devices such as arterial lines.

In maternity, midwives and HCAs chart observations using MEWS, not NEWS. An urgent call for a medical registrar to review a woman will therefore be via a RED MEWS call directly to bleep 5807.

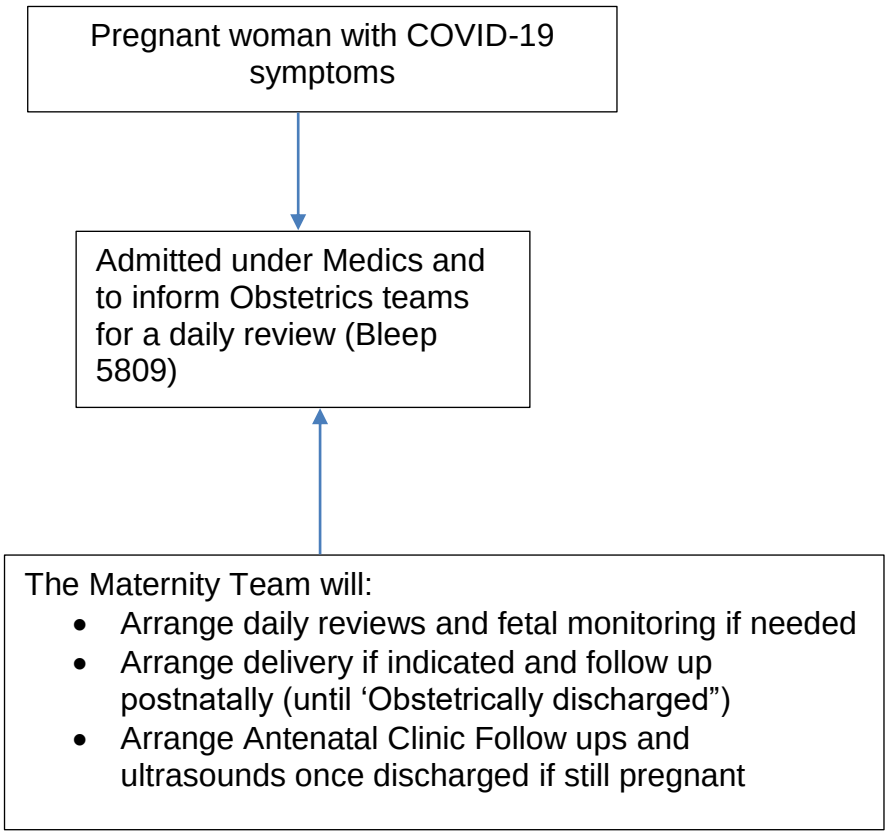
There is an anaesthetic registrar assigned 24/7 to labour ward. Consideration should be given to calling them (bleep 5809) whilst awaiting a review from the medical registrar.

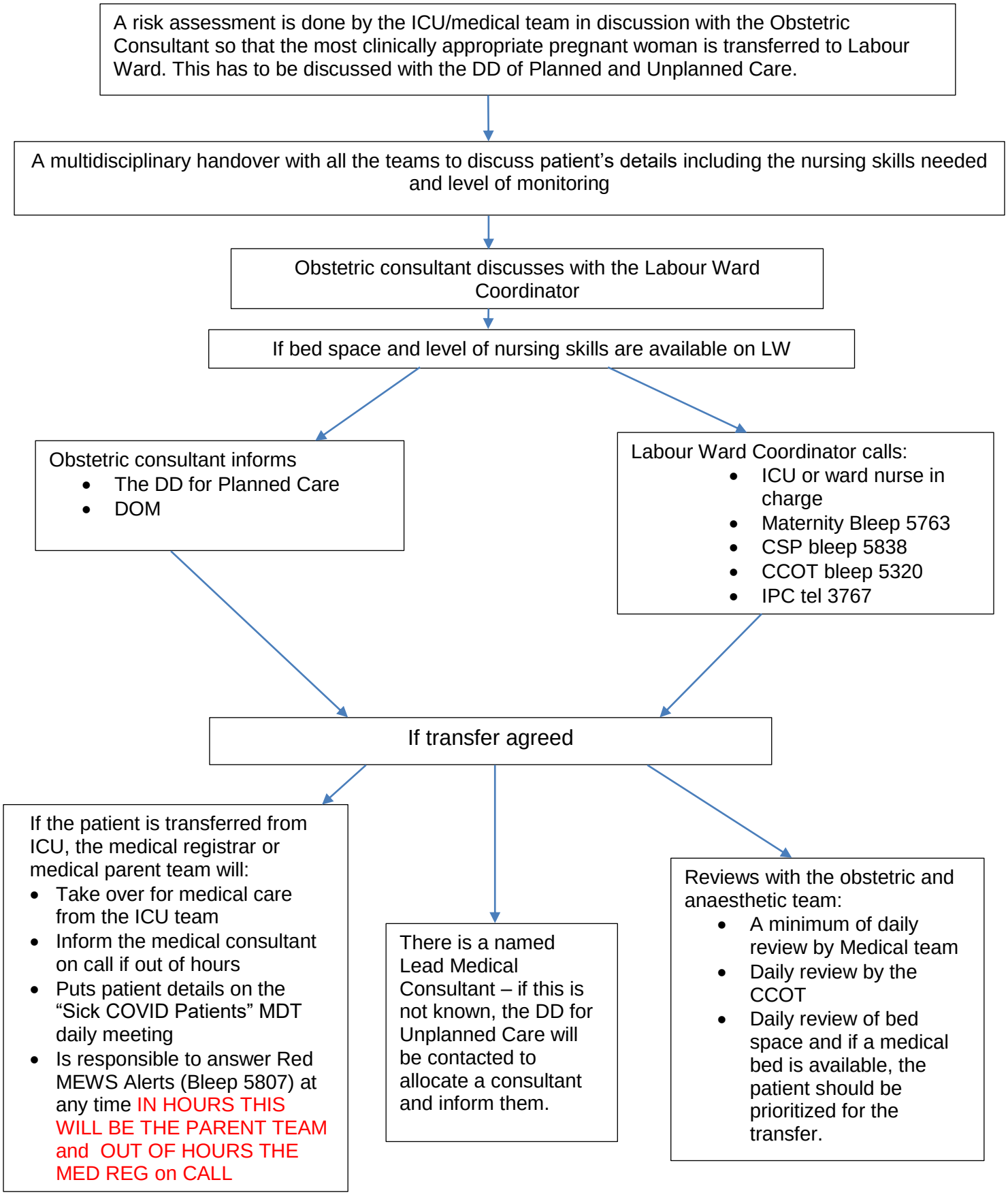
There is currently a daily "Sick COVID-19 patient" MDT. The ICU or medical team must ensure that the maternity patient is on the list on the day of transfer so that her management is kept under daily review.

The SOP for COVID-19 adult care should be followed in maternity with special attention to VTE prophylaxis and glucose control if she is on steroids. An early GTT should be considered if there are the usual recognised maternal risk factors for GDM. A growth scan should be arranged for 2 weeks after admission with follow up in a consultant ANC if discharged.

On labour ward, these patients remain under the care of medicine.

Appendix 1: Flowchart for Pregnant Women/Birthing People with Symptoms of COVID-19 Needing Admission



Appendix 2: Flowchart for Process of Transfer of A Pregnant Patient from ICU/Medical Ward to Labour Ward

This is ONLY in the event that there are no beds in the medical wards: